

Treatment Recommendations

- **Permethrin 5% cream** is effective and requires application on three successive days (level of evidence Ib; grade A recommendation).
- **Synergized pyrethrins**, available as a foam preparation in some countries, are as effective as permethrin 5% cream (level of evidence IIa; grade B recommendation).
- **Lindane** is no longer recommended due to its potential to cause neurotoxicity.

Crusted Scabies

- Topical scabicide (permethrin 5% cream or benzyl benzoate 25% lotion) applied daily for 7 days, then twice weekly until cure, **and**
- Oral ivermectin 200 micrograms/kg on days 1, 2, and 8.
- For severe cases with persistent live mites on follow-up skin scrapings, additional ivermectin doses may be required on days 9 and 15, or days 9, 15, 22, and 29 (level of evidence IV; grade C recommendation).

Post-Treatment Itch

Post-treatment itch should be managed with repeated application of emollients. Oral antihistamines and mild topical corticosteroids may also be beneficial.

Special Situations

- Permethrin is safe to use during pregnancy (level of evidence III; grade B recommendation) and lactation, and is licensed for children aged 2 months and older.
- Benzyl benzoate and sulfur are considered safe in pregnancy (level of evidence III; grade B recommendation).
- Ivermectin should not be used during pregnancy or in children weighing less than 15 kg.
- Malathion has not been studied in pregnant women. Animal studies show no risk, but animal data may not reliably predict human response. Inappropriate use of agricultural-grade malathion for human infestations can cause acute toxicity (level of evidence IV; grade C recommendation).

Mass Population Treatment (level of evidence Ib; grade A recommendation)

- Recommended for scabies control in endemic areas (e.g., remote communities, displaced populations) and during outbreaks in closed settings such as nursing homes or prisons.
- All individuals should be treated regardless of symptoms.
- Oral ivermectin is easier to administer than topical agents, facilitating large-scale treatment.
- A single dose of oral ivermectin (200 micrograms/kg) is effective.
- Ivermectin may not sterilize scabies eggs; a second dose after one week improves efficacy. A second dose is recommended (level of evidence Ib; grade A recommendation), though the impact of this on long-term control requires further evaluation.

- Drug resistance to scabicides, including permethrin and ivermectin, is an emerging concern. The effect of mass treatment programs on resistance development needs further study.

Follow-Up

A follow-up visit 2 weeks after treatment completion is recommended for microscopy-based test of cure (level of evidence IV; grade C recommendation).

Partner Management

- Patients should avoid close contact until they and their sexual partners have completed treatment (level of evidence IV; grade C recommendation).
- Infestation in children is rarely due to sexual abuse and is more commonly from close non-sexual contact.
- Sexual partners from the past 2 months should be assessed and offered epidemiological treatment (level of evidence IV; grade C recommendation).

Prevention and Health Promotion

- Risk of scabies can be reduced by limiting the number of sexual partners and practicing strict personal hygiene in crowded environments (e.g., avoiding sharing of underwear, clothing, bedding, and towels; minimizing skin-to-skin contact).
- Condom use does not prevent transmission.
- No additional preventive measures have been proven effective.

Auditable Outcome Measures

- Patients with scabies should be invited for a follow-up visit: target 95%.
- Suspected scabies cases should receive a recommended treatment regimen: target 95%.